



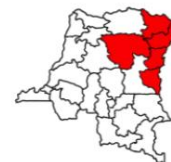
EBOLA PRESIDENTIAL FORCE TASK 17

Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°116/MVEBDB/08/09/2026

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HIGHLIGHTS (last 24 hours)

As of September 7, 2026, 71 new confirmed cases of Ebola Virus Disease (EVD), including 31 community deaths, were recorded in the provinces of Ituri (41 cases), North Kivu (27 cases), and Haut-Uélé (3). However, 10 deaths intra-CTE have also been reported. No new health zones have been affected in the last 24 hours.

ACCUMULATIVE CASE 6,757	ACCUMULATIVE DEATH CONFIRMED 3,267	LETHALITY 48.3%	PATIENTS IN ISOLATION/CTE 813	ACCUMULATIVE HEALED 1,590	TRACKING RATES CONTACTS (Of the Day) 88.3%
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Affected Provinces and Affected Health Zones



6 Provinces Affected

Haut-Uélé, Ituri, North Kivu, South Kivu, Tshopo & Bas Uélé



61 out of 151 health zones affected (40.4%)



Summary of key points from the last 24 hours

As of September 7, 2026, **71 new confirmed cases** of Ebola Virus Disease (EVD) were reported, including **31 community deaths**. The provinces reporting cases were **Ituri (41 cases), North Kivu (27), and Haut-Uélé (3)**. However,

We regret to report **10 deaths within the CTE (Treatment and Treatment Center)**, including 9 in Ituri and 1 in North Kivu, bringing **the number of deaths for the day to 41**.

In total, **27 patients** were declared cured after obtaining two negative results for Ebola Virus Disease (21 in Ituri, 4 in Haut Uélé and 2 in Nord-Kivu) in the last 24 hours.

The DRC has recorded **6,757 confirmed cases, including 3,267 deaths**, representing a case fatality rate of **48.3%**. Since the start of the epidemic, Ituri province has remained the epicenter, accounting for **80.0%** of confirmed cases reported across the six affected provinces. Furthermore, it accounts for **57.7%** of new cases.

Fortunately, no new health zones have been affected in the last 24 hours. Thus, **61 out of 151 health zones (40.4%)** remain affected in the six affected provinces. Ituri has 28 out of 36 affected health zones, followed by North Kivu (16/34), Haut-Uélé (6/13), Tshopo (7/23), South Kivu (1/34), and Bas-Uélé (3/11).

Table 1. Distribution of confirmed cases and deaths by affected province, as of September 7, 2026.

Province	New confirmed cases (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	41	5406	2,450	45.3%	28/36 (77.8%)
North Kivu	27	1066	697	65.4%	16/34 (47.0%)
Haut-Uélé	3	256	107	41.8%	6/13 (46.1%)
Tshopo	0	22	9	40.9%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas Uélé	0	4	3	75.0%	3/11 (27.3%)
Total	71	6,757	3,267	48.3%	61/151 (40.4%)

Table 2. Distribution of confirmed cases and deaths by province and health zone, as of September 7, 2026.

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	5406	2450	45.3%	41	15	9	24
Adja	11	1	9.1%				0
Aru	8	6	75.0%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	176	39	22.2%				0
Boga	2	2	100.0%				0
Bunia	1502	471	31.4%	14	7		7
Damascus	30	9	30.0%				0
Drodro	15	7	46.7%				0
Fataki	78	30	38.5%				0
Gethy	7	3	42.9%				0
Kambala	2	0	0.0%				0
Kilo	33	12	36.4%				0
Komanda	117	79	67.5%	4	4		4
Lita	245	122	49.8%	3	1		1
Logo	13	5	38.5%				0
Lolwa	25	7	28.0%				0
Mahagi	4	1	25.0%				0
Mambasa	17	7	41.2%				0
Mandima	42	23	54.8%	1	0		0
Mangala	295	141	47.8%	5	0		0
Mongbwalu	638	291	45.6%	4	1		1
Nia-Nia	227	115	50.7%	1	0		0
Nizi	709	273	38.5%	2	0		0
Nyankunde	125	36	28.8%				0
Rimba	11	4	36.4%				0

Rwampara	992	347	35.0%	5	2		2
Tchomia	62	25	40.3%	2	0		0
To ventilate	N / A	387	N / A	0	0	9	9
North Kivu	1066	697	65.4%	27	15	1	16
Blessed	181	135	74.6%	5	3		3
Good	9	4	44.4%				0
Butembo	211	146	69.2%	3	2		2
Goma	1	0	0.0%				0
Kalunguta	18	9	50.0%				0
Katwa	468	302	64.5%	11	5	1	6
Kayna	1	0	0.0%				0
Kyondo	23	15	65.2%				0
Lubero	6	4	66.7%				0
Mabalako	10	7	70.0%				0
Manguredjipa	4	3	75.0%	1	1		1
Masereka	15	6	40.0%				0
Musienene	100	54	54.0%	6	4		4
Mutwanga	2	2	100.0%				0
Oicha	9	6	66.7%				0
Vuhovi	8	4	50.0%	1	0		0
Haut-Uélé	256	107	41.8%	3	1	0	1
Boma Mangbetu	29	14	48.3%				0
Gombari	3	1	33.3%				0
Isiro	92	37	40.2%				0
Pawa	46	23	50.0%	3	1		1
Rungu	2	1	50.0%				0
Wamba	84	31	36.9%				0
Tshopo	22	9	40.9%	0	0	0	0
Bafwasende	2	0	0.0%				0
Kabondo	4	1	25.0%				0
Lubunga	1	0	0.0%				0
Makiso-Kisangani	10	5	50.0%				0
Mangobo	3	2	66.7%				0
Tshopo	1	0	0.0%				0
Wanie-Rukula	1	1	100.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	4	3	75.0%	0	0	0	0
Buta	1	1	1				0
Viadana	1	0	0.0%				0
Ganga	2	2	100.0%				0
Total	6757	3267	48.3%	71	31	10	41

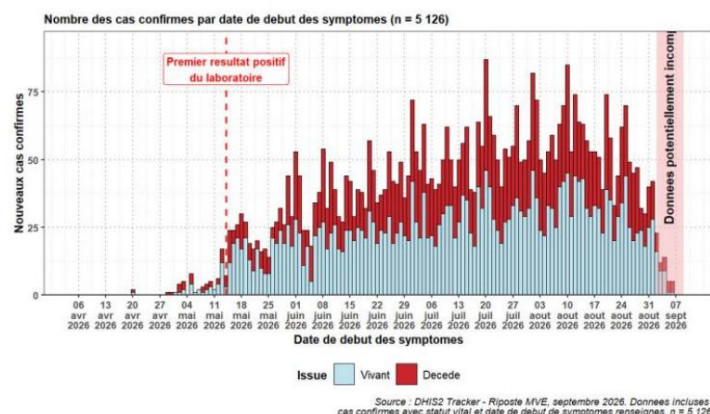
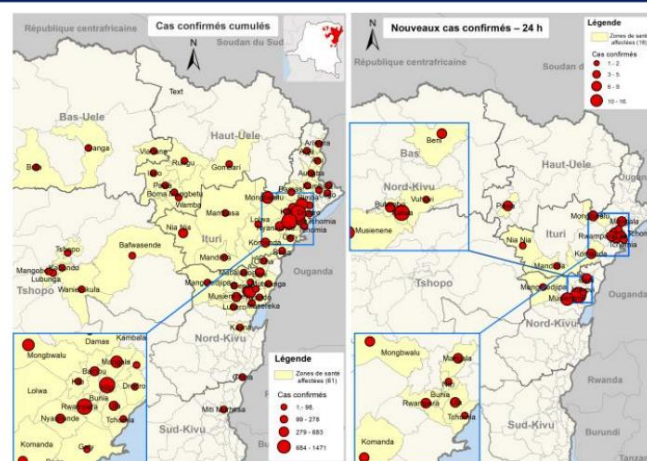
*These are the confirmed deaths that occurred in the various CTEs in the province and are being broken down to be classified by health zone.

NA: Not Applicable.

** These deaths were recorded in the various CTEs of the Ituri province and are currently being assessed.

Table 3. Status of alerts notified by province, as of September 7, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	13	4	17	11	4	2	0	15	11
Bas Uélé	15	0	15	4	0	11	0	4	0
Ituri	655	75	730	123	57	294	7	180	105
North Kivu	597	41	638	74	41	509	0	115	31
South Kivu	11	0	11	8	0	3	0	8	6
Tshopo	97	0	97	6	1	85	0	7	1
Total General	1,388	120	1,508,226	103	904	7	329	154	

Figure 1. Evolution of cases by date of symptom onset over the last 21 days**Figure 2. Spatial distribution of confirmed cases (24h and cumulative) by health zone.**

1. COORDINATION

1.1. Main Actions

- Monitoring and support of response actions in all affected provinces.
- Discharge ceremony of 22 recovered patients from the Isiro CTE under the patronage of the Provincial Minister of Health and preparation, with the DGRHU, followed by the professional reintegration of a recovered patient.

2. RESPONSE ACTIONS BY PILLAR

1.1.1. Epidemiological surveillance

1.1.2. Main Actions

During the course of the day on September 7, 2026, 1,508 alerts were reported in 6 Of the provinces, 1,240 (82.2%) could be verified. All 329 alerts validated as suspected cases were investigated (100%) and 154 of them were transferred to CT/CTE (Table 3).

Of the 24,719 contacts to be followed up for the day of September 7, 21,359 of them were seen, representing a daily proportion of **88.3%**, above the threshold of 85%; **90.7% in Ituri (12,054/13,289)**, 87.3% in North Kivu (8,580/9,833), 70.4% in Bas Uélé (107/152), 69.5% in Tshopo (196/282) and 67.7% in Haut Uélé (422/623).

1.1.3. Challenges

Insufficient teams dedicated to contact tracing activities make contact tracing ineffective, particularly in Bas Uélé.

1.2. Update on checkpoint and point of entry (PoC/PoE) activities

1.2.1. Main actions

Seven (7) alerts were notified to the various PoE/PoCs as of September 7, 2026, including four in Ituri, two in Bas Uélé, and one in North Kivu. In Ituri, three alerts were intercepted at the Mudzibala PoC (AS Nzere, ZS de Bunia), and all were validated.

Table 4. Status of key indicators for PoC/PoE by province, as of September 7, 2026

Indicators	Ituri	North Kivu	South Kivu	Tshopo	Haut-Uélé Bas-Uélé	Overall	
Report completeness	90% (54/60)	100% (19/19)	55.5% (5/9)	39% (7/18)	64.0% (7/11)	36% (9/25)	71.1% (101/142)
% of functional strategic PoE/PoC	100%	100%	100%	100%	100%	100%	100%
Number of people who switched to PoE/PoC % of	146370	87959	11312	3693	11,288	4215	253549
travelers screened	98.00%	98.18%	99.00%	99.00%	98.00%	98.8%	98.50%
% of travelers who washed their hands	98.00%	98.18%	99.00%	97.5%	98.00%	98.8%	99.09%
% of travelers made aware	99.00%	98.18%	100.00%	100.00%	98.00%	95.8%	99.25%
Number of alerts notified	4	1	0	0	0	2	7
Number of validated live alerts (suspected cases)	3	0	0	0	0	0	3
Number of bodies intercepted today; % of suspected	0	1	0	0	0	0	1
cases transferred to CT/CTE	66.7%	0%	-	-	-	-	66.7%
% of lifeless bodies swabbed/secured	-	100%	-	-	-	-	100%
% of suspected cases investigated within 24 hours	100%	100%	-	-	-	100%	100%

1.2.2. Challenges

Operationalization of PoE/PoC remains suboptimal: completeness remains low in Bas-Uélé (34.6%, 9 reports received out of 26 expected), in Sud-Kivu (55.5%, 5/9) and in Haut-Uélé (64.0%, 7/11) and internet connection not provided in 4 PoC; persistent insecurity hindering the commissioning of the Avakubi PoC (Ituri); strike following non-payment of PoC providers (Nzebi, Galay, Pluto and Bey); in Tshopo, only 18 of the 30 planned PoCs are activated.

1.3. Laboratory

1.3.1. Main actions

- **Ituri : 41 new positive results** (26 living and 15 deaths) out of 279 new samples received and analyzed (**positivity: 14.7%**).
- **North Kivu : 27 new positive results** (12 living and 15 dead) out of 174 samples analyzed (**positivity of 15.5%**).
- **Haut-Uélé: 3 new positive results** (2 living and 1 death) out of 19 samples analyzed (**positivity: 15.8%**).

- **Bas-Uélé: 3 samples** received and tested (3 live), **all came back negative.**
- **Tshopo: 6 samples** received and tested (6 live), **all came back negative.**

1.4. Infection Prevention and Control (IPC/EDS)

1.4.1. Main actions

- **In Ituri**, 25 of the 70 planned infection control rings were opened (35.7%), 73 of the 73 identified households were decontaminated (100%), and 10 of the 11 health facilities were decontaminated (91.0%); 60 infection prevention and control (IPC) kits were distributed to households in the Mongbwalu health zone. Regarding the blood and health surveillance (BHS), 92 alerts were recorded, 79 BHS were carried out (85.9%), and 67 of the 79 bodies were swabbed.
- **In North Kivu**, 31 of the 34 planned infection control rings were opened (91.2%), 13 of the 26 identified households were decontaminated (50.0%), and 30 of the 42 health facilities were decontaminated (71.4%), without the distribution of kits to households; 21 health facility assessments were carried out with an average score of 50.8%; IPC/WASH training was provided to the management teams of the health zones in Butembo and Katwa; and training began for 100 hygiene workers from Butembo, Katwa, and Musienene with UNICEF support. The Health Data System (HDS) component received 51 alerts (22 were postponed from the previous day), 50 HDS assessments were carried out, and 51 bodies were swabbed (100%).
- **In Tshopo**, provision of IPC inputs in 8 health establishments in the health zones of Makiso-Kisangani and Kabondo; replacement of a mattress in an accommodation establishment (Kabondo health zone, 21st case); monitoring and support of 14 health establishments in 5 health zones (Makiso-Kisangani, Mangobo, Tshopo, Kabondo and Lubunga) with the briefing of 125 providers on IPC measures; 2 rings opened in Kabondo and Mangobo.
- **In Haut-Uélé**, all 6 expected rings were opened (100%), 27 households decontaminated out of 30 identified (90.0%) and 5 out of 5 FOSEA (100%), 10 ESS supported and 64 providers briefed in the 6 affected health zones; 5 EDS alerts were received, 4 EDS carried out and 5 bodies swabbed.

1.4.2. Challenges

- **In Ituri** : Low operation of the rings (35.7%, 25/70) and insufficient decontamination teams in Fataki, Mangala, and Komanda; lack of mobility in Bambu and Mandima; non-implementation of activities due to supply shortages in Mangala and Mandima. **In North Kivu** : Insufficient decontamination of identified households (13/26), and teams still need to be activated and trained in Biena, Masereka, Kyondo, Mabalako, and Vuhovi. **In Tshopo** : Insufficient IPC inputs in social and solidarity economy (SSE) facilities, untrained service providers, and a malfunctioning incinerator at the Makiso-Kisangani General Referral Hospital. **In Haut-Uélé** : persistent unavailability of household kits limits support, and only 8 of the 30 SSE facilities under monitoring have a functional triage unit (26.7%).
- For the EDS, non-swabbed bodies in Ituri (Mandima and Fataki), due to the absence of teams, lack of inputs and 8 in North Kivu (Kalunguta and Vuhovi) due to insufficient means of transport; persistence of the lack of mobility for the EDS teams of the ZS of Kilo, Nia-Nia, Mandima and Fataki.

1.5. Holistic care

1.5.1. Main actions

- **In Ituri**, there were 77 new admissions to the various dedicated facilities, compared to 88 discharges, including 21 recoveries. Thus, 473 patients are hospitalized out of 978 beds, representing an occupancy rate of 48.4%; **433/457 (95%)** patients received hot meals; **14/14 (100%)** children are receiving LNPE/SLM at the CTE, **3/3 (100%)** children are receiving LNPE/SLM at the Crèche, and **28/28 (100%)** children are receiving LNPE/SLM in the community; analysis of the ORACLE-BDBV protocol for clinical trials of a treatment candidate (INRB) continues.

- **In North Kivu**, 74 new admissions and 91 discharges were recorded, including 2 recoveries; 254 patients are hospitalized expressing an overall bed occupancy rate in CTE/CT is in oversaturation (115.5%; 220 beds available).
- **In Haut Uélé**, there were 11 new admissions compared to 6 discharges (4 recovered). Thus, 62 patients are hospitalized out of 128 beds, representing an occupancy rate of 48.4%.
- **In Tshopo**, two new admissions were recorded and three discharges (no one recovered). Four (4) patients are in isolation out of 25 beds, representing an occupancy rate of 16.0%.
- **In South Kivu**, 6 new admissions during the day, 20 patients are in isolation (Occupancy rate: 80.0%).

1.5.2. Challenges

Care still partially substandard and limited capacity: **in North Kivu**, 101 of the 254 hospitalized patients (40%) remain in facilities that are not Ebola Treatment Centers (ETCs). **In Ituri**, the ETCs/CTs in Nizi, Mambasa, Lolwa, and Kilo are at capacity with suspected cases. **In Haut-Uélé**, the Pawa ETC lacks hygiene facilities, and the Isiro ETC lacks space, water, and a constant supply of electricity. **In Tshopo**, the lack of safe blood and limited technical resources are hindering care. **In Bas-Uélé**,

The absence of CTE/CT and ambulance limits the isolation of cases.

1.6. Vaccination

1.6.1. Main actions

- **In Tshopo**, 3,000 doses of ERVEBO were received out of 11,000 expected, 500 doses were thawed and 100 doses were deployed to Kabondo, 300 to Makiso-Kisangani and 100 to Mangobo.
- **In Ituri**, a meeting was held to discuss the composition, recruitment, and briefing of the actors in charge of vaccinating PPLs in Bunia.

1.6.2. Challenges

Insufficient doses available in Tshopo (3,000 received out of 11,000 requested), lack of mobility for contact tracing (3 vehicles required for the mobile strategy), and the Bafwasende vaccination site remains inactive. **In Bas-Uélé**, the stockout of ERVEBO in Buta has led to a temporary suspension of vaccinations for PPL and TPL; micro-plans are lacking in the 6 priority health zones (Aketi, Ango, Bili, Buta, Ganga, and Viadana).

1.7. Continuity of Care

- **In Bas-Uélé**, 16 high-risk contacts remain under post-exposure prophylaxis in the health zones of Buta (10) and Ganga (6), the province having not transmitted an updated report in the last 24 hours.
- **In Tshopo**, evaluation of the Malkia health center (lack of drinking water), followed by the launch of the drug working group, and the delivery of essential generic medicines from health zones to health areas.
- **In Haut-Uélé**, a ceremony was held to unload 22 patients who had recovered from the CTE in Isiro under the patronage of the Provincial Minister of Health, and preparation was made with the DGRHU for the professional reintegration of one patient; 6 other patients are awaiting their unloading in Isiro.
- **In Tshopo**, 3 new high-risk contacts were placed under post-exposure prophylaxis, including 1 frontline provider, and 2 recovered individuals joined the response teams.

1.8. Risk communication and community engagement

1.8.1. Main actions

- **In Ituri**, 29 members of civil society and community leaders were briefed on Ebola prevention measures, with an emphasis on dignified and safe burials and support from World Vision; 17,258 home visits reached 29,803 people, 566 educational talks reached 9,915 people, 4 community debates reached 45 people and 24 mass awareness campaigns reached 1,841 people; 53 contacts were pre-listed around confirmed cases in Komanda.
- **Haut Uélé** : Advocacy to His Excellency the Provincial Minister of Communication for the support of the activities of the response against Ebola by the Media throughout the province; 8 mass activities reached 2,395 people, 76 home visits reached 456 people and 60 educational talks reached 433 people.
- **In Tshopo**, 4,270 people were sensitized, including 2,880 students from 4 schools in Kisangani, 796 people during home visits in 156 households in the health zones of Mangobo and Tshopo, 392 during a parade and 46 during a community dialogue around the CTE of Météo under construction; 2 feedbacks were resolved and 1 incident lifted.

1.8.2. Challenges

Persistent community resistance: **In Tshopo**, an attack against teams was recorded in the Bethsaida health district (Mangobo health zone) amid rumors of an "Ebola business"; 11 contacts are refusing follow-up in the Matete health district (Mangobo health zone); and a refusal to provide samples was resolved through negotiation in Kumbakisaka. **In Haut-Uélé**, a refusal to participate in the census persists in Tely (Isiro health zone); rumors are circulating that patients are being held in tents at the Isiro General Referral Hospital, and posters and leaflets have been destroyed. **In Ituri**, resistance to the Ebola Health Distribution Teams (EDTs) has been reported in Bunia, Nizi, Nia-Nia, and Aru. **In North Kivu**, a confirmed case escaped after confirmation in Masereka, and a family in Munzambaye (Butembo health zone) is refusing decontamination of their household.

1.9. Mental Health and Psychosocial Support (MHPSS)

1.9.1. Main actions

- **In Ituri**, 19 of the new confirmed cases and 147 of the 195 suspected cases received SMSPS interventions; 73 test results were announced, including 19 positive results, 16 PPL supported and 43 households assisted, 49 children and 35 recovered, followed up in the community.
- **In North Kivu**, 29 people among the new confirmed cases and 165 confirmed cases under follow-up were taken care of, as well as 67 new suspects; 105 announcements of results including 39 positive, 56 PPL supported, 67 households accompanied, 11 of the 14 recovered followed.
- **In Tshopo**, 2 confirmed cases and 4 suspected cases received psychological support, 3 results were announced all negative, 22 caregivers and 11 PPLs were supported and 44 people were reached by psychoeducational sessions.
- **In Haut-Uélé**, 5 new confirmed cases and 36 confirmed cases under monitoring benefited from SMSPS interventions, as well as 5 new suspected cases and 32 suspected cases under monitoring; 20 test results were announced, including 5 positive results; 89 caregivers, 78 people with disabilities, and 4 family members received support; and the 22 recovered patients were discharged from Isiro with assistance.

1.9.2. Challenges

In Ituri, there is poor coverage of SMSPS interventions around the latest confirmed cases in Ituri (Komanda and Lita) and North Kivu (Biena, Manguredjipa, Kayna, Lubero, Masereka, Kalunguta, Mabalako, Mutwanga, and Vuhovi), where childcare facilities are lacking in 10 affected health zones. **In Haut-Uélé**, there is insufficient staff and means of transport (Boma Mangbetu and Wamba), and an absence of SMSPS providers in Tshopo (Bafwasende, Wanie-Rukula, and Isangi).

1.10. Logistics

1.10.1. Main actions

- **In Ituri**, 16 of the 32 organized patient transfers were successful (50%) and 25 patients were evacuated; food support was provided to 433 of the 457 hospitalized patients (95%), 14 children under LNPE/SLM at the CTE (100%), 3 in nursery and 28 in the community; 2 ambulances were mobilized (1 in Bambu and 1 in Nizi) and food needs remained in Kilo (5), Lolwa (13), Mambasa (4) and Mandima (2).
- **In North Kivu**, 14 calls were received for 14 trips carried out and all were successful (100%) and 9 ambulances were mobilized out of the 11 available (82%); 93 of the 93 patients (100%) and 41 accompanying persons received a hot meal, 2 confirmed unstable patients received a specific diet at the CTE in Beni and 129 frontline providers were fed; 139 parcels of inputs were received.
- **In Haut-Uélé**, 46 patients, 70 accompanying persons and 25 PPLs benefited from three hot meals; receipt of 4 refrigerators for the laboratories of Isiro and Wamba, 150 pallets and 2 tarpaulins for the UMS tents; delivery of PPE to the ZS of Pawa and donation of 1,500 liters of fuel by MEMISA.
- **In Tshopo**, monitoring of the construction and deployment of the CTEs (Technical and Economic Centers) in Météo and Bafwasende of 3 trucks of medicines (11 tonnes) to Lubunga.

1.10.2. Security

1.10.3. Main actions

- **In North Kivu**, six community policing activities were carried out in the Beni sub-coordination area: support and security coverage for IPC teams at the Amani Health Center (Bundji Health District), the Shamamba Health Center and the Sifa Dispensary (Benengule Health District), the Paida Hospital (Paida Health District), and in a household in the Rwangoma Health District for decontamination, as well as for the EDS team for the burial of three confirmed cases in the Bundji and Masiani cemeteries; all these activities were fully covered. Security support was also provided in Oicha for the feasibility study of an isolation site and a standardized Ebola Treatment Center at the General Referral Hospital.

1.10.4. Challenges

In North Kivu, there is a lack of a permanent vehicle for the security team and a lack of financial incentive for the agents; there is a need to systematically inform the security team before any field visit.

1.11. PSEA (Prevention of Sexual Exploitation and Abuse)

1.11.1. Main actions.

- **In Ituri**, briefing of 61 service providers (RECO, PRESICOSA and registered nurses) from the Mambasa health zone, including 41 women and 20 men, and continuation of the display of messages and distribution of PSEAH leaflets in the health zones of Bunia, Aru, Mongbwalu, Mambasa and Fataki.
- **In Haut-Uélé**, briefing of frontline people on the importance of reporting to the toll-free numbers 495555 and 151 and community awareness in the health zones of Isiro (160 community relays including 108 men and 52 women), Wamba (476 people), Boma Mangbetu (70), Gombari (55 students) and Rungu (5), i.e. 745 people sensitized; 5 of the 6 health zones affected (83.3%) now know the reporting mechanism.

1.11.2. Challenges

- Still insufficient coverage of PSEA activities, with only 13 of the 36 health zones in Ituri (36.1%) aware of the reporting mechanism; lack of mapping of stakeholders and refusal of some partners to align; insufficient mass awareness materials; lack of computer kits and office supplies for the pillar.

KEY MESSAGES

- Consolidation of progress in epidemiological investigations (**100% of suspected cases investigated**) and a stability in the daily proportion of contact tracing (**88.3%**) above the threshold of 85.0%.
- However, continued reporting in the provinces of Ituri, North Kivu and Haut-Uélé highlights sustained transmission of the disease at the level community). The province of North Kivu presents a worrying profile well beyond its characteristic high lethality, **38.0%** of new confirmed cases were reported there compared to **47.6%** the day before.
- As of September 7, 2026, **71 new confirmed cases**, including **31 community deaths and 10 intra-CTE deaths**, were recorded in the last 24 hours, bringing the national total to **6,757 confirmed cases and 3,267 death (case fatality rate: 48.3%)**.
- Ituri remains the epicenter of the epidemic, accounting for **80.0** % of cumulative confirmed cases and **57.7%** of new cases reported in the last 24 hours.
- Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to quickly interrupt transmission chains: Rapidly increase reception capacities in CT/CTE in North Kivu.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.



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